

1. Administrative & Study Identification

Official Title: A PHASE 3, RANDOMIZED, OPEN-LABEL STUDY OF PF-06821497 (MEVROMETOSTAT) IN COMBINATION WITH ENZALUTAMIDE COMPARED WITH ENZALUTAMIDE OR DOCETAXEL IN PARTICIPANTS WITH METASTATIC CASTRATION RESISTANT PROSTATE CANCER PREVIOUSLY TREATED WITH ABIRATERONE ACETATE (MEVPRO-1) **Brief Study Title:** A Study to Learn About the Investigational Medicine Called PF-06821497 (Mevrometostat) in Men With mCRPC Who Were Previously Treated With Abiraterone Acetate for Prostate Cancer (MEVPRO-1). **Short Title/Acronym:** MEVPRO-1 **Protocol Number:** C2321014 **NCT Number:** NCT06551324 **Posting ID:** 1063265666330242001 **Sponsor / Company Name:** Pfizer **Trial Status:** RECRUITING **Investigational Product:** PF-06821497 (Mevrometostat) + Enzalutamide **Phase of Development:** Phase 3 **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the radiographic progression-free survival (rPFS) of the combination of PF-06821497 (Mevrometostat) plus enzalutamide versus physician's choice of either enzalutamide or docetaxel. **Secondary Objectives:** To evaluate Overall Survival (OS), Objective Response Rate (ORR), Duration of Response (DoR), safety, pharmacokinetics, circulating tumor DNA (ctDNA), and patient-reported outcomes.

2.2 Background & Rationale To address resistance to androgen receptor (AR) pathway inhibitors (ARPI; e.g., abiraterone, enzalutamide) in metastatic castration-resistant prostate cancer (mCRPC). This resistance may be driven by the preservation of AR signaling through alternative pathways, including epigenetic modifiers like Enhancer of Zeste Homolog 2 (EZH2). Mevrometostat is a potent and selective small molecule EZH2 inhibitor. Combining ARPI with mevrometostat represents a promising treatment approach to overcome hormone therapy resistance, delay cancer progression, and improve outcomes in patients who have progressed after first-line treatment with abiraterone acetate.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Physician's choice of Enzalutamide or Docetaxel) **Blinding:** Open-label **Randomization:** 1:1 Stratified

3.2 Planned Enrollment & Duration Target \$\$\$: Approximately 600 male participants **Number of Sites:** Multi-center (Global)
Estimated Study Start: 21-Oct-2024 **Estimated Primary Completion:** 13-Jul-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Men aged ≥ 18 years with histologically or cytologically confirmed adenocarcinoma of the prostate without small cell features. 2. Metastatic disease in bone documented on bone scan, or in soft tissue documented on CT/MRI scan. 3. Progressive disease in the setting of surgical or medical castration with evidence of disease progression on treatment with abiraterone acetate in the mCSPC setting or first line mCRPC setting is required. 4. Eastern Cooperative Oncology Group (ECOG) performance status 0 - 2, with a life expectancy of at least 6 months as assessed by the investigator.

4.2 Exclusion Criteria 1. Prior treatment for prostate cancer at any stage with any cytotoxic chemotherapy, radioligand therapy (i.e., ^{177}Lu -PSMA-617, radium 223), androgen receptor signaling inhibitors (ARSi) including enzalutamide, apalutamide, darolutamide, poly ADP-ribose polymerase (PARP) monotherapy, or other systemic anti-cancer treatment, with standard protocol exceptions (e.g., first-generation antiandrogen agents discontinued prior to first dose; docetaxel treatment allowed for mCSPC if no signs of failure/progression occurred during treatment or within 3 months of completion). 2. Known or suspected brain metastasis, active leptomeningeal disease, or clinically significant history of seizure. 3. Any medical or psychiatric condition including recent (within the past year) or active suicidal ideation/behavior, or laboratory abnormality that may make the participant inappropriate for the study. Clinically significant cardiovascular disease, known history of active inflammatory gastrointestinal disease, chronic diarrhea, or previous gastric resection or lap-band surgery. 4. Previous administration with an investigational product within 30 days or 5 half-lives preceding the first dose of study intervention (whichever is shorter). 5. Inadequate organ function.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - *Investigational Arm:* Mevrometostat 875 mg twice daily (BD) taken by mouth with food + Enzalutamide 160 mg once daily (QD). - *Comparator Arm:* Physician's choice of Enzalutamide 160 mg once daily (QD) by mouth OR Docetaxel (ATC Code: L01CD02; Trade names: Docefrez, Taxotere) 75 mg/m² intravenously every 21 days. **Route of Administration:** Oral (Mevrometostat, Enzalutamide) and Intravenous (Docetaxel). **Duration of Treatment:** Until objective

disease progression, unacceptable toxicity, or subject withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing disease symptoms or treatment-related adverse events. **Prohibited:** Other systemic anti-cancer treatments, unapproved investigational therapies, and medications known to interfere with the metabolism of study drugs.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Scans (CT/MRI/Bone), Labs
Baseline	Day 1	Randomization, First Dose, Safety Labs
Interim	Every 3-4 Weeks	Safety Labs, Efficacy Assessment, PRO Questionnaires, Investigational Product Dispensation
End of Study	Follow-up	Off-Treatment Safety Follow-up, Ongoing Survival Follow-up

(Note: Routine imaging scans to evaluate rPFS are conducted at specified pre-defined intervals during the interim phase)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Radiographic Progression-Free Survival (rPFS), defined as the time from the date of randomization to first objective evidence of radiographic progression as assessed in soft tissue per Response Evaluation Criteria in Solid Tumors (RECIST) v1.1 or in bone per Prostate Cancer Clinical Trials Working Group 3 (PCWG3) guidelines by Blinded Independent Central Review (BICR), or death, whichever occurs first.

7.2 Secondary & Exploratory Endpoints - Overall Survival (OS) - Objective Response (ORR) - Duration of Response (DoR) in measurable soft tissue disease - Safety and Tolerability Profiles (Adverse Events) - Pharmacokinetics and circulating tumor DNA (ctDNA) markers - Patient-Reported Outcomes (e.g., mobility, pain, quality of life)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard continuous collection. Special monitoring focus on diarrhea, dysgeusia, and anemia, which were previously noted in early-phase trials of mevrometostat. Patient-reported outcomes via routine questionnaires will record self-assessed severity. **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness to the sponsor. **Data Monitoring Committee (DMC):** An Independent

Data Monitoring Committee (IDMC) will review safety and efficacy data systematically.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy assessments; Safety Population for adverse event reporting. **Sample Size Justification:** \$N \approx 600\$ participants designed to provide robust statistical power to evaluate significant improvements in rPFS and the key secondary endpoint, OS. **Handling of Missing Data:** Standard survival analysis methodologies will be applied. Time-to-event endpoints will be summarized utilizing Kaplan-Meier analyses, and stratified log-rank P-values, HRs, and 95% CIs will be estimated using a stratified Cox proportional hazard model.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard site monitoring (onsite and remote) assessing investigator compliance, protocol adherence, and safety reporting. **Audit Trail:** eCRF entry by trial site staff with centralized data validation, query generation, and resolution via data clarification forms.

11. Study Identifiers & Governance

Primary ID: C2321014 **Registry Number:** NCT06551324 **Posting ID:** 1063265666330242001 **Sponsor/Lead Organization:** Pfizer **Study Title (Acronym):** MEVPRO-1 **Official Regulatory Title:** A PHASE 3, RANDOMIZED, OPEN-LABEL STUDY OF PF-06821497 (MEVROMETOSTAT) IN COMBINATION WITH ENZALUTAMIDE COMPARED WITH ENZALUTAMIDE OR DOCETAXEL IN PARTICIPANTS WITH METASTATIC CASTRATION RESISTANT PROSTATE CANCER PREVIOUSLY TREATED WITH ABIRATERONE ACETATE (MEVPRO-1)

12. Clinical Framework (Prostate Cancer Specific)

Primary Condition: Metastatic Castration-Resistant Prostate Cancer (mCRPC) **Disease Terminology (Metastatic):** Defined as the clinical or pathologic observation of a tumor extension from its original site of growth to another anatomic site (MeSH Code: D009369; SNOMED ID: 263650009). **Diagnosis Threshold:** Castration testosterone levels ≤ 50 ng/dL and radiographic/biochemical progression on abiraterone acetate. **Medical Methodology:** Epigenetic Modification (EZH2 inhibition) + Androgen Receptor

Signaling Inhibitor (ARSi) **Optimization Target:** Overcoming hormone therapy resistance to delay radiographic progression.

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathway combining oral targeted therapy with systematic central radiological review
Education Protocol (HCP): Site initiation visits and periodic protocol retraining (including RECIST/PCWG3 reporting and EZH2 safety profiling)
Education Protocol (Patient): Counseling on treatment adherence, side-effect reporting (e.g., diarrhea, dysgeusia), and completion of self-rated health questionnaires
Quality Audit Mechanism: Independent central imaging review and routine source data verification

14. Observation & Follow-Up Schedule

Total Duration: Estimated ~4 Years (Study Completion estimated Oct-2028)
Onsite Visit Cadence: Every 3 weeks (Docetaxel arm) or every 4 weeks (Mevrometostat/Enzalutamide arm)
Remote Monitoring Frequency: Routine query reviews through centralized clinical database
Checklist Review Interval: Routine eCRF and Patient-Reported Outcomes (PRO) collection matching treatment cycles

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				